Mobile health applications for mental health care: Review and analysis of functionalities and technical features

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Abstract

Background: Mental health disorders, including depression and anxiety, have become leading causes of disability worldwide. Digital mental health interventions, particularly mobile applications, constitute a promising strategy for improving access to mental health care.

Methods: A Systematic Literature Review (SLR) protocol was adopted to perform the selection, data extraction and functional evaluation of available mental health apps on iOS and Android platforms. The analysis of the functionalities and technical features of the apps selected was performed according to a 38-items assessment questionnaire developed on the basis of scientific literature of mental health interventions and a preliminary analysis of available mental health apps.

Results: A total of 44 mental health apps were retrieved from the app repositories of the iOS and Android platforms. The results of the functional content analysis show that the mental health apps selected relatively achieved varied scores owing to the complexity and the breadth of mental health care interventions.

Conclusions: The present study helps in identifying areas related to mental health care that require further endeavors to be properly addressed. It also provides guidance for developers to build more complete and effective mental health apps.

Keywords: mental health apps; mobile health; systematic review; mHealth; digital mental health; app evaluation

1. Introduction

Mental health, as defined by the World Health Organization (WHO), exists when people achieve their complete potential while handling their daily activities and work duties and supporting their communities (World Health Organization, 2018). The Diagnostic and Statistical Manual of Mental Disorders (DSM-5) characterizes mental health as a dynamic state encompassing emotional, psychological and social aspects that impact cognitive processes, mood and behavioral responses (American Psychiatric Association, 2013). Mental health exists beyond the absence of illness because it provides vital support for achieving complete wellness and performing daily activities.

Mental disorders, which include depression and anxiety, have become the leading causes of disability throughout the world. The World Health Organization estimates that depression impacts over 280 million people and anxiety disorders affect a similar number of individuals globally (World Health Organization, 2023). The current state reveals a severe lack of mental health treatment access because more than 75% of low- and middle-income country residents cannot access mental health services (Patel et al., 2018). The main obstacles to receiving care include stigma and discrimination, high treatment costs, insufficient healthcare providers and geographic barriers.

The existing service gaps have found a potentially transformative solution through digital mental health services, commonly referred to as e-mental health. The field encompasses web-based therapy systems, telepsychiatry services and mobile health applications designed to enhance mental health care delivery (Torous et al., 2018). These tools work to provide mental health care

services that extend beyond conventional treatment settings through affordable, scalable solutions.

The COVID-19 pandemic precipitated a significant transformation in digital mental health services. The rapid adoption of telehealth services and digital interventions occurred as a result of lockdowns, health system constraints and increasing mental health concerns (Wind et al., 2020). What began as an emergency measure evolved into a sustained shift toward hybrid and remote mental health service delivery models.

Mobile applications provide specific benefits which distinguish them from other digital interventions. The platforms attract users who may avoid traditional therapy through their provision of anonymity, continuous availability, personalization and accessible pricing (Firth et al., 2017a). The applications commonly incorporate features including mood tracking, cognitive-behavioral therapy (CBT) exercises, relaxation techniques and progress monitoring.

The proliferation of mental health applications on mobile devices has generated multiple significant challenges. Many apps lack clinical validation, operate without regulatory oversight and fall short on data privacy protections, which raises both ethical and practical concerns (Larsen et al., 2019; Huckvale et al., 2015). Research indicates that peer-reviewed trials and transparent security practices support fewer than 1% of available apps.

The content of various apps relies on evidence-based frameworks including Cognitive Behavioral Therapy (CBT), Acceptance and Commitment Therapy (ACT) and Mindfulness-Based Stress Reduction (MBSR) to establish credibility (Firth et al., 2017a; Levin et al., 2012). These frameworks deliver structured treatment approaches which replicate standard

therapeutic methods to assist users in developing emotional regulation skills, modifying thought patterns and activating positive behaviors.

Experts advocate for the development of comprehensive evaluation frameworks to assess both clinical effectiveness and user experience. Mental health researchers and Human-Computer Interaction (HCI) experts concur that applications require evaluation for usability, cultural appropriateness, safety features and clinical efficacy (Bakker et al., 2016; Torous & Roberts, 2017). Digital mental health programs do not achieve their complete potential in the absence of adequate evaluation and regulatory oversight.

The main purpose of the present study is to review and analyze features and functionalities of the pool of apps for mental health care that are currently available in the two most widely used smartphone application stores: Apple App Store and Google Play Store. In the realm of digital health, previous studies have scrutinized the functionalities of available mobile applications for specific conditions such as postpartum depression (Zhang et al., 2017) and pregnancy monitoring (Bachiri et al., 2016). To the best of our knowledge, this is the first study that has been conducted to thoroughly analyze the features and functionalities of mobile applications targeting comprehensive mental health care. This review addresses the following research questions:

- RQ1: Which are the common types of mental health care apps selected?

- RQ2: Which are the predominant characteristics of mental health care apps?

- RQ3: To what extent do the mental health care apps comply with the items analyzed?

- RQ4: Are user ratings proportionally linked to the apps' characteristics?

- RQ5: Which mental health disorders are most frequently targeted by these apps?

The remainder of the present paper is structured as follows: Section 2 outlines the research questions and describes the methodology used to search for, select and analyze the functionalities of mental health care apps. Section 3 answers the research questions and discusses the findings. Section 4 explores the limitations and the threats to validity of the present study. Section 5 draws some concluding remarks and gives a summary of future work.

2. Method

The search for mental health care apps was addressed through the use of systematic literature review (SLR) methodology (Brereton et al., 2007) that ensures the accuracy and completeness of the search and retrieval process. Prior to performing the search for candidate mental health care apps, a protocol which outlines the different steps constituting the SLR process was developed in accordance with a set of guidelines delineated by the Preferred Reporting Items for Systematic reviews and Meta-analysis (PRISMA) (Moher et al., 2009) statement. The PRISMA guidance is said to improve the quality of reporting in systematic reviews and provide substantial transparency in the selection process. The review of mental health care apps was completed through the following steps: (1) framing the research questions, (2) defining search strategy, (3) identifying eligibility criteria for the selection process, (4) determining elements for analysis and (5) formulating assessment questions. Each of these steps was accomplished by two authors

independently. Any disagreements and discrepancies were discussed until consensus was reached.

2.1. Framing the Research Questions (RQs)

To rigorously address the purpose of the review, a set of five research questions was identified by the authors (Table 1). These research questions were informed by the scientific literature related to mental health interventions (Firth et al., 2017a; Levin et al., 2012; Marshall et al., 2020) and a preliminary analysis of available mental health care apps.

Table 1. Research questions.

Research Question (RQ)	**Rationale**
RQ1. Which are the common types of mental health care apps selected?	Determine the classification of the apps selected in terms of their purpose.
RQ2. Which are the predominant characteristics of mental health care apps?	Identify the common functionalities and features among mental health care apps.
RQ3. To what extent do the mental health care apps comply with the items analyzed?	Examine the comprehensiveness of the apps selected in terms of functionalities and features.

RQ4. Are user ratings proportionally linked to the apps' characteristics?	Investigate the relationship between users' perception of the quality of the apps selected and the relevance of the functional content of these apps.
RQ5. Which mental health disorders are most frequently targeted by these apps?	Identify the mental health conditions addressed and evaluate the approaches used.

2.2. Defining Search Strategy

The leading app stores namely, Google Play Store and Apple App Store (Statista, 2025) were considered to perform the search for candidate mental health care apps. The PICO criteria (Population, Intervention, Comparison and Outcome) (Stone, 2002) were used to define a search string that ensures maximum coverage. The Population considered was that of individuals seeking mental health support. The Intervention consisted of free and paid apps which are intended to support users' mental well-being. Given that the primary focus of the present study was not comparative and did not seek to find evidence comparing existing mental health care apps, the Comparison criterion was disregarded. With respect to the last criterion, all the aspects and outcomes of mental health care apps were of interest in the present study, of which managing mental health conditions and promoting emotional well-being were the mostly considered.

The following terms: 'mental health' OR 'anxiety' OR 'depression' OR 'CBT' OR 'mindfulness' OR 'therapy' OR '(mental health OR anxiety OR depression) AND support' OR '(therapy OR mood) AND tracking' were therefore identified and were primarily applied to the title and

description of the apps using the search tools in both app stores. The search process took place in January 2025.

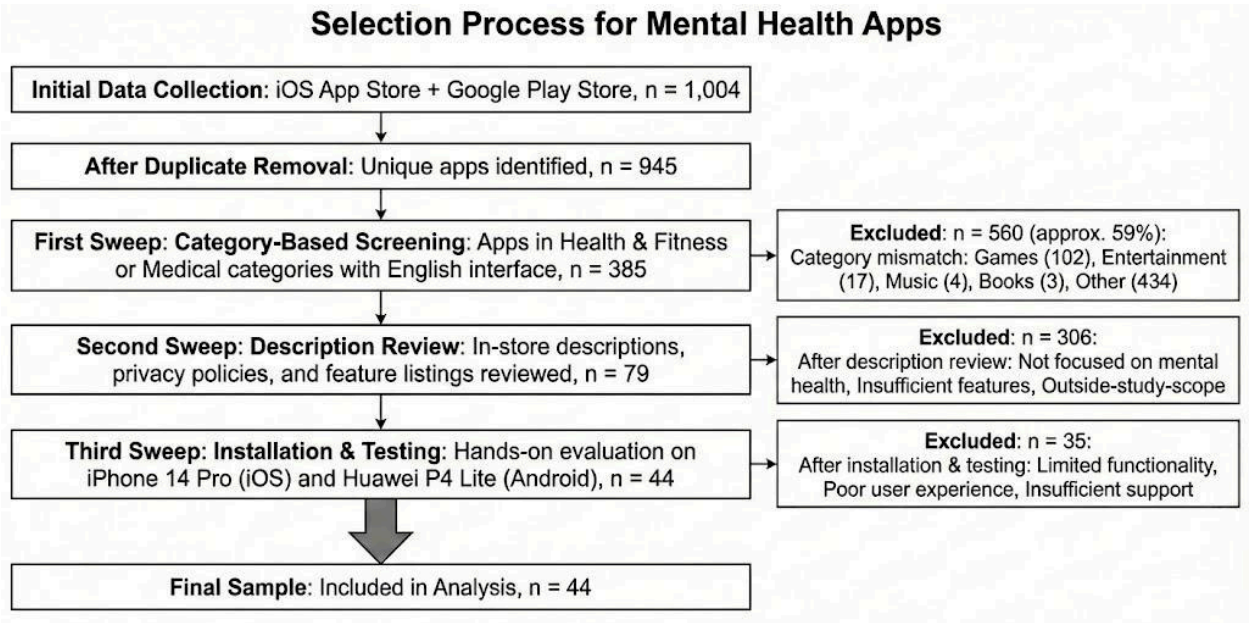


Fig. 1. PRISMA flowchart showing the selection process of mental health care apps

2.3. Identifying Eligibility Criteria for the Selection Process

To identify the candidate mental health care apps and discard the disqualifying ones, a set of eligibility criteria was defined. Apps fulfilling all the inclusion criteria (IC) were included in the present study. These criteria are the following:

IC1. Free or freemium apps obtained using the pre-defined search terms and available in the reviewed app stores. Apps offering some free content alongside paid features were considered to ensure accessibility.

IC2. Apps pertaining to categories related to health namely, Health & Fitness or Medical. This criterion ensures focus on health-oriented applications rather than general wellness or entertainment apps.

IC3. Apps that are primarily focused on any aspect related to mental health care for users. Apps had to demonstrate clear focus on supporting individuals with mental health concerns, whether through therapy tools, educational content, or self-monitoring features.

IC4. Apps required to have a clear clinical foundation, with features grounded in recognized therapeutic methods such as Cognitive Behavioral Therapy (CBT), Acceptance and Commitment Therapy (ACT), or mindfulness-based interventions.

Further exclusion criteria (EC) were applied to the apps identified throughout the selection process to eliminate:

EC1. Duplicates. This exclusion criterion aims to discard duplicates either within the same app store and apps available for both platforms under the same name and have identical content.

EC2. Apps that are not designed with an English interface.

EC3. Apps that were malfunctioning upon installation or could not be accessed. This criterion excludes all apps presenting any kind of bug or error during or after installation that prevent authors from conducting an appropriate analysis of content.

EC4. Apps that do not offer any free content and require immediate subscription. This ensures that assessment can be conducted on accessible features without financial barriers.

2.4. Determining Elements for Analysis

Scientific literature and mental health care apps available in the app stores were reviewed to identify the main components for the evaluation of features and functionalities of the mental health care apps selected. Two components were determined: (1) app characteristics comprising the common features of any app; and (2) data items including the fundamental functionalities and features to be included in a mental health care app at a functional and a technical level.

2.4.1. App Characteristics

The app characteristics that should be retrieved from the app store for each app selected include: the full name, the operating system (Android or iOS), cost (free, freemium, in-app purchases), the app link in the app store or app website (if available), date of the latest update, user ratings, number of raters and country of origin of app developer.

2.4.2. Data Items

A thematic template was designed containing the data that should be extracted in regard to the functional content of the mental health care apps selected. This template was piloted using a sample of published papers, standards and guidelines related to mental health interventions as shown in Table 2; agreement was reached about six blocks for a comprehensive mental health care app among the first three authors.

Table 2. Relevant sources of information for each block of items.

Data Items	**Relevant Sources of Information**
Clinical Content (Evidence-Based Interventions)	✓ Studies on CBT-based mobile interventions [7,20] ✓ Research on mindfulness-based digital interventions (Khoury et al., 2015; Goldberg et al., 2018) ✓ ACT framework applications in digital health (Levin et al., 2012; Brown et al., 2016)
Self-Monitoring and Reminders	✓ Studies on mood tracking effectiveness (Kauer et al., 2012) ✓ Research on reminder systems in mental health apps (Ahmed et al., 2018)
Therapy, Counseling, and Peer Support	✓ Studies on digital therapeutic relationships (Hollis et al., 2015) ✓ Research on peer support in mental health apps (Baker & Yang, 2018)
Crisis Management and Emergency Aid	✓ Guidelines on crisis intervention in digital platforms (Stanley & Brown, 2012) ✓ Studies on safety planning tools (Bryan et al., 2018)
Education and Personalized Content	✓ Research on psychoeducation delivery (Baumel et al., 2019) ✓ Studies on personalization in mental health apps (Schueller et al., 2017)

App Technical Features	✓ Studies about security and privacy concerns in mobile health applications [32,33] ✓ Usability guidelines for mental health applications [34,35] ✓ Accessibility standards for digital health (Zapata et al., 2018)

Clinical Content (Evidence-Based Interventions). This category encompasses features based on recognized psychological frameworks that deliver structured interventions. Cognitive Behavioral Therapy (CBT) represents a methodology that guides users to identify maladaptive thoughts and behaviors through tools including guided journaling, thought records and reflective prompts. In many implementations, the app presents a specific situation and invites the user to note their immediate reaction in order to explore the emotional weight it carries and then examine the thinking patterns behind it, often gently revealing distortions such as rigid black-and-white reasoning or catastrophizing. Based on this reflection, the app suggests more balanced alternatives that help reframe the original thought [7,20]. Beyond this, some apps introduce behavioral tasks that encourage users to re-engage with daily activities, particularly important for individuals experiencing depression, through small enjoyable actions when withdrawal has become a pattern. Others help users navigate practical stressors by offering structured problem-solving sequences, guiding them step by step from defining a challenge to deciding on an action. Some apps include brief educational explanations that clarify how thoughts, emotions and behaviors influence one another, thereby supporting users in building greater self-awareness over time.

Mindfulness-based practices constitute another pillar of evidence-based content, helping users develop present-moment awareness through features including timed meditations and breathing exercises. The goal of mindfulness practice is to reduce emotional reactivity and assist individuals in regulating their emotions with adaptive coping mechanisms (Khoury et al., 2015; Goldberg et al., 2018). These practices are often structured as brief daily sessions that guide users through focused attention on the breath or bodily sensations. The simplicity of these exercises allows them to be integrated into daily routines, making them accessible even to users with limited time or experience. Regular use has been shown to decrease rumination and improve emotional regulation, particularly among individuals experiencing symptoms of stress, anxiety or depression. Some applications also incorporate reminders throughout the day that prompt users to briefly pause and check in with their mental state; this approach has been linked to increased self-regulation and reduced impulsivity. As these strategies build the habit of present-moment attention, they support the development of non-judgmental awareness, which represents a core protective factor in mental health.

Acceptance and Commitment Therapy (ACT) represents another behavioral approach that promotes setting goals aligned with one's values and encourages making small daily commitments to achieve these goals. It also emphasizes emotional acceptance through interactive reflection (Levin et al., 2012; Brown et al., 2016). In digital formats, ACT-based tools often begin with guided exercises that help users clarify what truly matters to them in domains such as relationships, personal growth or health, before encouraging them to take small, concrete steps in that direction, even in the presence of difficult emotions. This process is grounded in the ACT principle of psychological flexibility, which refers to the ability to remain present and act

according to one's values despite internal discomfort. Many apps include reflective journaling prompts or brief audio sessions that support users in observing their thoughts without judgment and separating their identity from their inner experiences. These elements aim to reduce experiential avoidance and have shown promising outcomes for improving anxiety, depression and overall well-being.

Self-Monitoring and Reminders. This category includes tools that help users track emotional states and reflect on changes over time. Mood and symptom trackers often appear as visual logs, where emotions are marked on scales or charts to reveal patterns that might otherwise go unnoticed (Kauer et al., 2012). Some apps allow users to link their mood to daily activities or events, helping them understand what influences their emotional state. Brief check-in prompts such as asking how a person feels in the moment or what has been stressful can encourage regular reflection without being overwhelming. To help users maintain consistency in their tracking, many apps use reminders to prompt self-checks, suggest coping activities or remind users to take their medication (Ahmed et al., 2018). These notifications are often adjustable, allowing users to choose the time, frequency or type of reminder that works best for their routine. When used regularly, these features have been shown to improve emotional awareness and build healthy routines that support long-term mental health.

Therapy, Counseling, and Peer Support. This category includes features that aim to replicate aspects of therapeutic relationships or offer direct mental health support (Hollis et al., 2015; Baker & Yang, 2018). Some apps provide structured therapy modules that follow evidence-based protocols, guiding users through a sequence of exercises that resemble the flow of clinical sessions. Others extend further by offering real-time interaction with licensed professionals,

either through chat or video, allowing users to receive tailored guidance within the app itself. Many platforms also create spaces for peer connection, such as anonymous group chats or moderated forums, where users can share experiences, offer support and feel understood. In some cases, these interactions extend to in-app community events or discussion threads, which help foster a sense of belonging among individuals navigating similar challenges.

Crisis Management and Emergency Aid. This category focuses on features designed to support users during moments of emotional distress or mental health emergencies (Stanley & Brown, 2012; Bryan et al., 2018). Some apps offer direct links or call buttons that connect users to crisis helplines, making it easier to reach professional help when needed. Others include tools such as emergency text alerts or location sharing, which can notify trusted contacts if the user feels unsafe. In addition to these reactive options, several apps provide proactive safety planning features. These often guide users through creating a personal plan that might include coping strategies, a list of warning signs or a set of contacts and resources to access during high-risk situations. Together, these functions aim to increase users' sense of safety and preparedness when navigating difficult moments.

Education and Personalized Content. This category highlights resources designed to help users better understand and manage their mental health (Baumel et al., 2019; Schueller et al., 2017). Information is often shared through brief articles, videos or audio segments that focus on topics such as stress management, emotional regulation or identifying symptoms. To make the experience more relevant, some apps adapt content based on user activity or offer assessments and personalized suggestions that match individual needs.

App Technical Features. Convenient, secure and easy-to-use mobile health apps have the potential to increase adherence and engagement with the health system as a whole (Martínez-Pérez et al., 2015). Security and privacy are considered as the most challenging risks encountered when developing mobile health solutions. Health-related apps, in particular, are dedicated to tracking, recording and managing personal health information (PHI) of users. PHI is extremely sensitive and needs to be highly protected through robust security and confidentiality mechanisms such as encryption and authentication methods (Sunyaev et al., 2015). Unauthorized access to health information represents a serious threat to privacy and security. Moreover, for users of mobile health apps, the existence of a privacy policy is an important baseline standard to understand why, where and how personal details and PHI will be collected, used, shared and protected.

Accessibility represents another app feature worth consideration. This includes offline functionality for select features, allowing continued use even without internet access. To accommodate different user preferences, apps may offer language selection, regional customization and accessibility tools such as adjustable text size or voice-guided navigation (Zapata et al., 2014; Moumane et al., 2016; Zapata et al., 2018). In some cases, users can export their data such as symptom logs or progress summaries for sharing with clinicians or for personal records. Clear onboarding instructions and built-in help sections are also frequently provided to assist users in navigating the app with confidence. The usability, for its part, is identified as one of the decisive factors in building efficient and intuitive apps. It relates to the ease with which users can complete their tasks in a specified context of use. Task structure and interaction style

should be optimal to minimize usability flaws. Usability aspects have a great impact on improving users' interaction with the apps, and ultimately, enhancing the overall user experience.

2.5. Formulating Assessment Questions

To extract relevant data for an optimal analysis of the functional content of the mental health care apps selected, a set of Yes/No questions was developed (Table 3). The developed questionnaire was drawn on the predefined data items and was validated by a licensed therapist who is highly acquainted with mobile technologies. The data obtained using the questionnaire were employed in answering the research questions. Each app was assessed separately for all the questions. The scoring was performed as follows: Yes (Y) = 1 point, the app offers the feature described in the question. No (N) = 0 points, the app does not offer the feature described in the question.

Table 3. Assessment questionnaire.

Data Items	**Questions**
Clinical Content (Evidence-Based Interventions) (CC)	1. Does the app include guided CBT exercises?
	2. Does the app offer mindfulness or meditation sessions?
	3. Are ACT-based tools (e.g., values clarification) included?

	4. Does the app feature relaxation techniques like breathing or muscle relaxation?
	5. Does the app provide psychoeducation about anxiety, depression or other disorders?
	6. Does the app explain the purpose and process of its therapeutic exercises?
Self-Monitoring and Reminders (SMR)	7. Does the app provide mood tracking functionality?
	8. Are there prompts for daily or regular emotional check-ins?
	9. Can users set reminders for medication or self-care tasks?
	10. Is the notification system customizable?
	11. Does the app allow tracking of triggers or context for mood entries?
	12. Can users view trends or summaries of their mental health data?
Therapy, Counseling, and Peer Support (TCS)	13. Does the app provide access to licensed professionals via chat or video?

	14. Are guided therapy modules available within the app?
	15. Does the app include peer support forums or group chats?
	16. Are there community events or discussion groups within the app?
	17. Does the app offer access to crisis counselors or mental health professionals?
	18. Are users able to receive encouragement or feedback from peers or moderators?
	19. Does the app provide access to a chatbot for simulated therapeutic conversations?
Crisis Management and Emergency Aid (CM)	20. Are crisis hotline links or buttons integrated into the app?
	21. Does the app allow emergency text or call activation?
	22. Can users share their location in emergencies?

	23. Is there a personalized safety plan or coping strategy tool available?
	24. Can users create a personalized emergency action plan?
Education and Personalized Content (EPC)	25. Does the app offer educational content on mental health topics?
	26. Is the content personalized based on user activity or needs?
	27. Are there goal-setting or value-reflection features?
	28. Can users track progress and receive feedback?
Technical Foundations (TF)	29. Is user authentication available (PIN, biometrics)?
	30. Does the app present a privacy policy?
	31. Does the app use data encryption or secure storage?
	32. Is offline access supported for key features?
	33. Are there multi-language options or regional settings?

	34. Does the app support accessibility features (e.g., font size, voice navigation)?
	35. Can users export their data (e.g., progress, symptoms)?
	36. Does the application implement intuitive navigation patterns?
	37. Does the application use a tone that is simple, informal and friendly?

3. Results and Discussion

This section presents and discusses the principal findings of the assessment conducted in the present study. Characteristics, functionalities and technical features of mental health care apps were retrieved and evaluated using a questionnaire of 38 items. The findings are sorted and grouped by research question.

3.1. Selection Process

The selection process of mental health care apps and the resulting number of apps after applying the inclusion and exclusion criteria is shown in Fig. 1. The app repository search identified 1004 free and paid apps, of which 94% (n=945) were excluded for being duplicates across platforms. Multi-platform publishing is currently considered a norm in mHealth app market, which justifies

the number of cross-platform apps initially identified. After this first screening, 385 apps were excluded for not pertaining to the categories defined in the present study, including 192 apps in Games, 33 in Books, 97 in Utilities, 64 in Music and 264 in Other categories. After reviewing the in-store description and screenshots of the remaining apps, 79 apps were retained for further eligibility assessment through installation and full review of their contents. Apps designed for Android were downloaded on a Huawei P30 Lite while iOS dedicated apps were downloaded on an iPhone 14 Pro. Upon installation and comprehensive testing, 34 apps were excluded: apps close to being discontinued, apps that were malfunctioning upon installation and apps that did not offer any free content and forced immediate subscription.

A total of 44 apps were selected for analysis of functional content, of which 38 apps were found in the Health & Fitness category and 7 apps originated from the Medical category. All selected apps offered free or freemium content, with 17 apps offering the possibility of in-app purchase and 20 offering either yearly or monthly subscription options. The English interface selection criterion biased the results such that predominantly USA-based apps were selected, with 30 apps originating from the United States, 2 from France and 1 from South Korea. The remaining apps' countries of origin were not clearly specified.

3.2. General App Characteristics

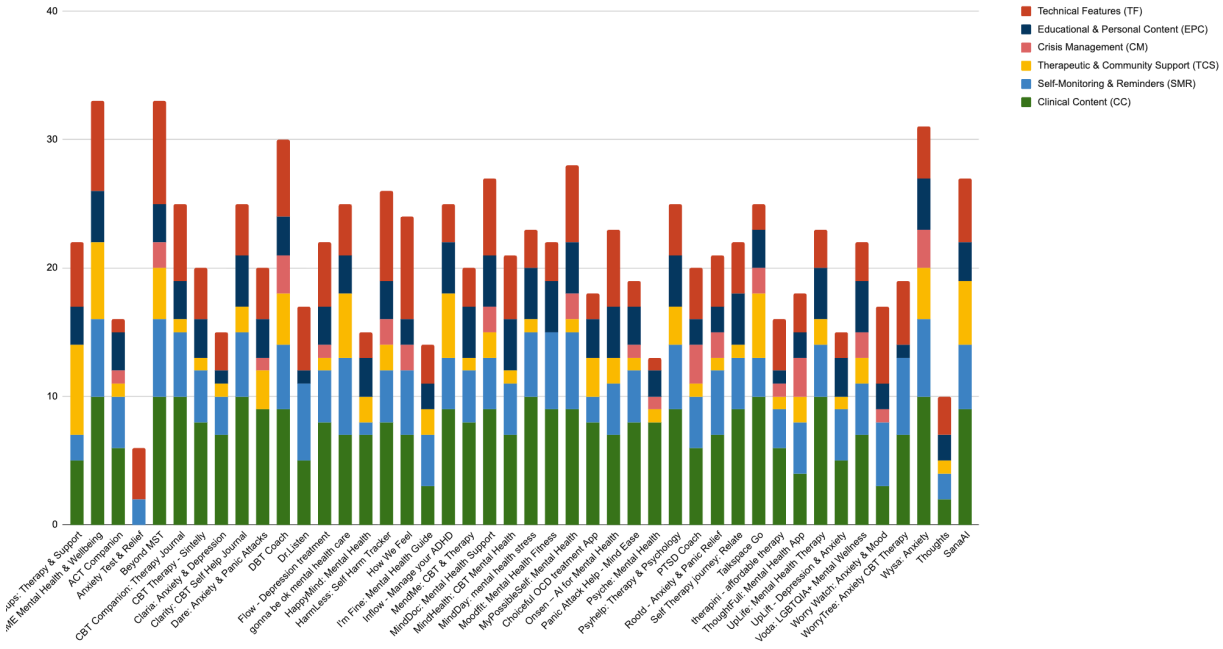
The overall characteristics of the apps selected are presented in Table A.1 (Appendix A). Among these, 38 apps were focused on the Health & Fitness category and 7 apps were considered medical. With regard to cost, 26 apps were completely free, 17 apps offered in-app purchases and 20 apps required subscription (monthly or yearly) for full access to features. With regard to the

country of origin, 30 apps originated from the United States while the remaining apps were developed in France, South Korea and other countries not clearly specified. This confirms the predominance of US-based developers in the mental health app market, though it may also reflect the English language criterion applied in the selection process.

3.3. Analysis of Functional Content

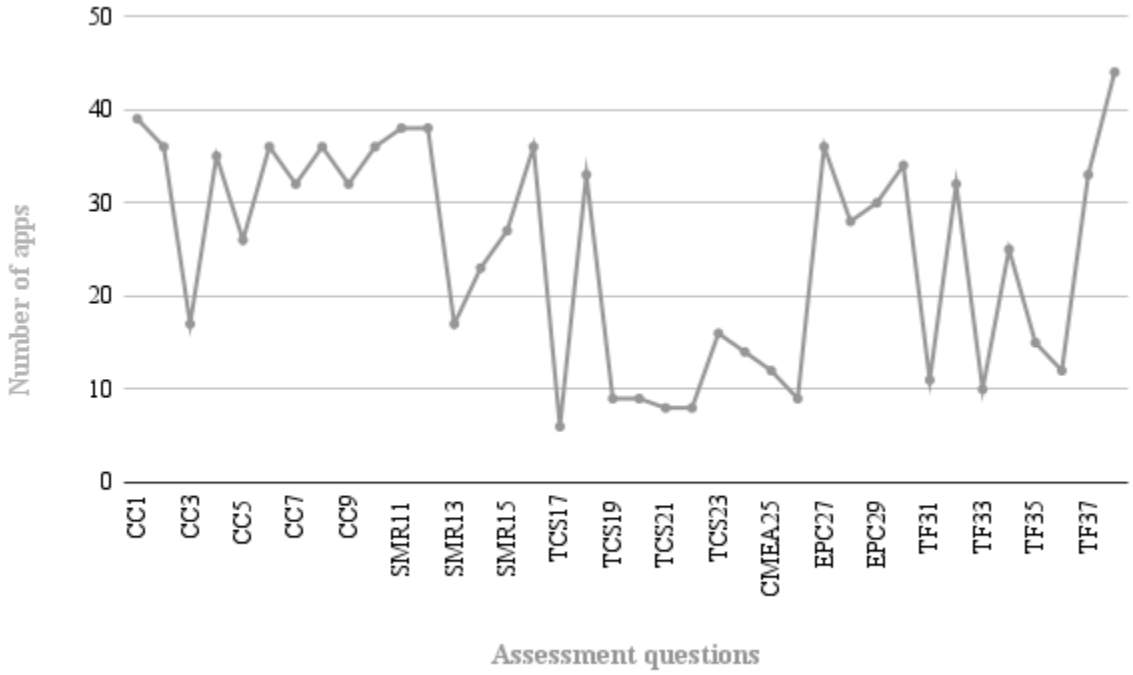
The results obtained from the assessment questionnaire are presented in Appendix B which describes the answers for each app and question. The total score percentage for each app evaluated which illustrates the assessment of its functional content in addition to the total percentage of each question referring to the number of apps fulfilling this question can also be depicted in Appendix B. For better analysis and interpretation, Figs. 2 and 3 illustrate these results. Fig. 2 presents the scores obtained by the 44 mental health care apps for each of the six blocks of assessment questions while Fig. 3 displays the number of apps fulfilling each of the 38 assessment questions.

Fig. 2. Assessment scores of the 44 mental health care apps across six functional blocks (Clinical Content, Self-Monitoring, Therapy/Counseling, Crisis Management, Education,Technical Foundations)



Based on these results, the maximum score (33 points, 87%) was obtained by AIME Mental Health and Wellbeing, followed by Wysa Anxiety which achieved 31 points (82%) and DBT Coach which obtained 30 points (79%). In contrast, Anxiety Test & Relief app obtained the lowest score with only 6 points (16%). With regards to the assessment questions, the highest score (98%) was reached by the question regarding informal and friendly writing style (TF38), followed by the question concerning guided CBT exercises (CC1) which attained a score of 87%. Moreover, the provision of mood tracking functionality (SMR7) and the ability to view trends or summaries of mental health data (SMR12) each achieved scores of 84%. The question related to access to crisis counselors or mental health professionals (TCS17) attained the lowest score (13%).

Fig. 3. Number of apps fulfilling each of the 38 assessment criteria



Prior to answering the research questions, it is worth mentioning that given the complexity of mental health care and the various elements it encompasses, the mental health care apps selected in the present study are likely to yield relatively varied scores with regard to the features they implement.

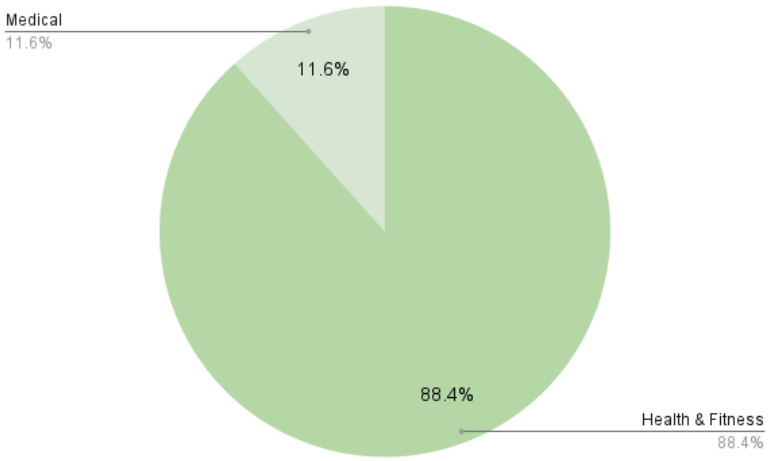
RQ1. Which are the common types of the mental health care apps selected?

Facilitated by technological advances, the range of mobile health solutions being designed is constantly expanding. This fundamental shift in the mHealth landscape has resulted in informed decision-making, improved self-management and increased efficiency in care delivery.

Moreover, mHealth apps have proven to enhance access to basic levels of healthcare and reduce barriers for both patients and healthcare providers. There is a particularly broad array of mHealth applications that serve different purposes including providing clinical interventions, remote monitoring, self-management tools, in addition to promoting healthy living and raising healthcare education and awareness.

Correspondingly, the mental health care apps selected fall into several main categories (Fig. 4). Approximately 40% of apps exclusively included features to track, monitor and record different aspects of mental health such as mood patterns, symptom severity and daily activities. These apps often incorporate visual mood logs and journaling features as seen in apps such as Daylio and MoodKit. Another prevalent category is the provision of structured therapeutic content and guided exercises based on evidence-based frameworks. Accounting for approximately 35% of the apps selected, therapeutic content is generally organized into modules addressing specific mental health concerns such as anxiety or depression, or providing comprehensive approaches that encompass multiple cognitive-behavioral techniques as in MindDoc and Sanvello apps. Of these therapeutic apps, many also offered features for tracking progress and recording personal reflections.

Fig. 4. Distribution of mental health apps by primary purpose category (Tracking 40%, Therapeutic Content 35%, Professional Support 20%, Mindfulness 5%)



A third category, representing approximately 20% of apps, was designed to provide immediate support through crisis management tools, peer support communities or access to licensed professionals. Apps such as 7 Cups and Talkspace exemplify this category by facilitating connections between users and trained listeners or licensed therapists. Lastly, a smaller proportion of apps focused primarily on mindfulness and meditation practices, offering guided sessions, breathing exercises and relaxation techniques to support general emotional well-being and stress reduction as in Headspace and Calm apps.

RQ2. Which are the predominant functional characteristics of mental health care apps?

To answer this research question, the results obtained through the assessment of the functional content of the apps selected were used. As shown in Appendix B, 87% of apps implemented guided CBT exercises (CC1), representing the most prevalent clinical feature among the apps analyzed. This characteristic was found in apps offering structured cognitive interventions that guide users through identifying and challenging maladaptive thought patterns. Besides allowing

users to engage with evidence-based therapeutic techniques, this characteristic helps in providing accessible psychological support as a function of user needs as in Sanvello and What's Up apps.

Another recurrently covered functionality is mood tracking (SMR7) which appeared in 84% of the apps analyzed. Mood tracking features allow users to log emotional states regularly, often through visual interfaces such as mood charts or emotion wheels. These tools enable users to identify patterns in their emotional experiences over time, which can facilitate greater self-awareness and inform coping strategies. Apps such as Daylio and eMoods exemplify comprehensive mood tracking systems that allow users to associate mood entries with activities, sleep patterns and other contextual factors.

The ability to view trends or summaries of mental health data (SMR12) was also implemented in 80% of the apps. This feature typically presents tracked information through graphs, charts or summary reports that visualize changes in mood, symptom severity or behavioral patterns over time. Such visualizations support users in recognizing progress, identifying triggers and understanding the effectiveness of coping strategies.

Thus, it may be observed that most of the mental health care apps analyzed in the present study prioritize evidence-based cognitive interventions and self-monitoring capabilities. Significant efforts are required to address other crucial aspects such as crisis management and access to professional support. With regards to the technical features, two usability aspects were largely present in the mental health care apps, namely informal writing style (TF38) and intuitive navigation patterns (TF37). Familiar writing style and intuitive layout accounted for 98% and 73% of the apps analyzed respectively. Another prevalent feature (TF30) which achieved a score

of 71% was that of including a privacy policy that discloses how the app collects, uses and shares information.

RQ3. To what extent do the mental health care apps comply with the items analyzed?

This research question approaches the main objective of the present study, which consists of measuring the compliance of the mental health care apps with the functional characteristics analyzed. Using the assessment results (Appendix B), each block of data items was separately investigated.

Clinical Content (Evidence-Based Interventions). The characteristics of evidence-based clinical content were addressed to a considerable extent in the majority of apps analyzed. Guided CBT exercises (CC1) appeared in 87% of apps, representing the most widely implemented clinical feature. Mindfulness or meditation sessions (CC2) were included in 80% of apps, while relaxation techniques (CC4) were present in 78% of the apps. However, ACT-based tools (CC3) were implemented in only 38% of apps, indicating that while CBT and mindfulness approaches are well-represented, other evidence-based frameworks remain underutilized. Psychoeducation about mental health disorders (CC5) was provided by 62% of apps, and explanations of therapeutic exercise purposes (CC6) appeared in 80% of apps. Overall, while most apps incorporate fundamental evidence-based techniques, there exists considerable variability in the depth and breadth of clinical content offered.

Self-Monitoring and Reminders. This block of characteristics was relatively well-implemented across the apps analyzed. Mood tracking functionality (SMR7) appeared in 84% of apps, and prompts for regular emotional check-ins (SMR8) were present in 84% as well. The ability to

view trends or summaries (SMR12) was implemented in 80% of apps. However, features for setting medication or self-care reminders (SMR9) appeared in only 38% of apps, and customizable notification systems (SMR10) were present in 51% of apps. The ability to track triggers or context for mood entries (SMR11) was included in 60% of apps. These findings suggest that while basic self-monitoring tools are prevalent, more sophisticated personalization features remain less consistently implemented.

Therapy, Counseling, and Peer Support. This category demonstrated the lowest implementation rates among all blocks analyzed. Access to licensed professionals via chat or video (TCS13) was available in only 13% of apps, while crisis counselor access (TCS17) appeared in only 13% as well. Guided therapy modules (TCS14) were more common, present in 73% of apps. Peer support forums (TCS15) appeared in 20% of apps, and community events or discussion groups (TCS16) were present in 20% as well. Encouragement or feedback from peers or moderators (TCS18) was available in 18% of apps, while therapeutic chatbots (TCS19) appeared in 36% of apps. These low implementation rates suggest that human connection and professional support remain significant gaps in the mental health app landscape.

Crisis Management and Emergency Aid. Features in this category were implemented in a minority of apps analyzed. Crisis hotline links or buttons (CM20) appeared in 31% of apps, while emergency text or call activation (CM21) was present in 27% of apps. Location sharing in emergencies (CM22) was implemented in only 24% of apps. Personalized safety plan tools (CM23) appeared in 20% of apps, and the ability to create emergency action plans (CM24) was present in 20% as well. These findings reveal a significant gap in crisis support features, which represents a substantial limitation given the potential severity of mental health emergencies.

Education and Personalized Content. Educational content on mental health topics (EPC25) was provided by 80% of apps, representing one of the most commonly implemented features. Content personalization based on user activity (EPC26) appeared in 62% of apps. Goal-setting or value-reflection features (EPC27) were present in 67% of apps, while progress tracking and feedback (EPC28) appeared in 76% of apps. Overall, educational and personalization features were relatively well-represented, though opportunities remain for more sophisticated adaptive content delivery.

Technical Foundations. User authentication options (TF29) were available in only 24% of apps, which raises concerns about data security on shared devices. Privacy policies (TF30) were present in 71% of apps, while data encryption or secure storage (TF31) was claimed by 71% of apps. Offline access (TF32) was supported in only 22% of apps. Multi-language options (TF33) appeared in 56% of apps, and accessibility features (TF34) were present in 33% of apps. Data export capabilities (TF35) were available in only 27% of apps. Intuitive navigation patterns (TF37) were implemented in 73% of apps, and informal, friendly tone (TF38) was present in 98% of apps. These findings indicate that while basic usability considerations are well-addressed, more advanced technical features such as offline access, authentication and data portability remain inconsistently implemented.

RQ4. Are user ratings proportionally linked to the apps' characteristics?

The answer to this question will help determine whether there is a relationship between the users' ratings for the apps selected and their corresponding assessment scores. One of the most interesting aspects of the mobile marketplaces (e.g., Google Play Store and Apple App Store) is

the emphasis on user feedback. These platforms allow users to review, comment and rate published apps between one to five stars according to their level of satisfaction with the app and its features. This process helps developers in obtaining insight into users' opinions about their apps. Recent research has found that star-ratings represent a crucial factor in the overall ranking of an app and can drastically affect user downloads.

Table C.1 (Appendix C) presents the ratings achieved by each of the apps selected along with the total number of raters as displayed in the app store at the time of data extraction. As portrayed in Table C.1, all apps that provided star-ratings were considered, including those available for both platforms. An independent-samples t-test was used to check the existing variances in the user ratings and the total number of raters for the mental health care apps between the Android and iOS platforms. The t-test did not yield a statistically significant difference between the two platforms neither for users' ratings $t(43) = -0.892$, $p = .377 > .05$, nor for the number of raters $t(43) = -1.156$, $p = .254 > .05$. Thus, it is likely assumed that the ratings of the apps being studied are similarly distributed to a normal distribution.

In an attempt to examine the compliance between app functional content and user satisfaction, we conducted a Pearson product-moment correlation coefficient. As shown in *Table C.2 (Appendix C)*, the coefficient ($r = -0.048$) denotes that the linear association between the two variables studied is very weak. Nevertheless, there is insufficient evidence to assume that this correlation exists given the statistically non-significant p-value ($p = .753$). The scatter plot in Fig. 5 illustrates the distribution of the mental health care apps analyzed around the line of best fit. It can be observed that some apps such as Dare and Anxiety Test & Relief yielded high user ratings (4.85 and 4.7 stars respectively) but obtained relatively low assessment scores (20% and 16%).

Furthermore, some apps such as AIME Mental Health and Wellbeing obtained high assessment scores (87%) but had limited user ratings data. This inconsistency between user feedback and the characteristics of the apps is perhaps due to several factors including the relatively low number of raters in certain apps, device-specific problems that could impact ratings, and the emphasis users may place on factors such as visual design, ease of use or emotional resonance rather than comprehensive functionality. Users are likely to appreciate apps that feel supportive or calming despite limited clinical depth, which negatively influences the correlation between ratings and functional scores.

Fig. 5. Scatter plot showing relationship between user ratings and assessment scores (r = -0.048, p = .753, n = 44)



RQ5. Which mental health disorders are most frequently targeted by these apps, and how are they addressed?

Analysis of the assessment data reveals that anxiety and depression were the most frequently addressed mental health disorders among the reviewed apps. These conditions appeared as primary targets in approximately 75% of the apps analyzed. For anxiety, apps commonly offered real-time calming techniques such as guided breathing exercises, grounding activities and meditation practices to help users manage acute stress or panic symptoms as in apps such as Rootd and Dare. Some apps extended further by including educational content about anxiety symptoms, triggers and cognitive patterns, promoting better understanding and long-term management strategies.

Depression was typically addressed through features encouraging self-reflection and behavioral activation. These included daily emotional check-ins, journaling exercises and goal-setting modules that helped users monitor and gradually reshape their mood and behavioral patterns. Many apps offered motivational support through progress visualization, achievement tracking and reminders for self-care activities strategies designed to counter depressive inertia and promote positive routines as seen in apps such as Moodpath and Sanvello.

A smaller but notable subset of apps targeted more specific disorders. Post-traumatic stress disorder (PTSD) was addressed in approximately 15% of apps, which included psychoeducation about trauma responses, coping strategy planners and in some cases screening tools based on validated instruments as in PTSD Coach. However, very few apps integrated comprehensive clinical pathways or therapist-guided trauma interventions. Obsessive-compulsive disorder (OCD) was addressed in approximately 10% of apps, often indirectly through habit tracking and exposure-like tasks, though without explicit mention of evidence-based protocols such as exposure and response prevention (ERP). For attention-deficit/hyperactivity disorder (ADHD), approximately 8% of apps incorporated time management aids, focus timers and behavioral coaching modules as in apps such as Inflow, though the therapeutic depth varied significantly.

Interestingly, while mood disorders and anxiety spectrum conditions were nearly universally covered, serious mental illnesses such as bipolar disorder, schizophrenia or eating disorders received minimal to no tailored content. This suggests a market tendency to cater to more prevalent or subclinical mental health concerns, possibly due to safety considerations, regulatory constraints or perceived user demand. Overall, the breadth of conditions targeted remains

relatively narrow, with most apps offering generalized mental wellness support rather than deeply personalized, disorder-specific interventions.

4. Threats to Validity

Although the present study was conducted with the aim to yield the most accurate and objective findings possible, there may have been some threats to the validity of the process. Several threats to validity must be considered when interpreting the findings of the present study.

First, content validity may be challenged by the subjectivity inherent in assessment criteria. Although the functional assessment was built on evidence-based frameworks (e.g., CBT, ACT, accessibility standards), the interpretation of whether an app fulfills each criterion relies on the evaluator's judgment. Without access to the app's backend or developer documentation, certain features such as encryption methods, clinical review processes or therapeutic progression algorithms may be misinterpreted or go undetected if they are not clearly presented to users. This risk is compounded by the absence of standardized disclosure requirements across app stores, where descriptions may be vague, marketing-oriented or outdated.

Secondly, construct validity may be weakened by the wide heterogeneity in app design and purpose. Some apps were designed purely for meditation practices, others for therapy support, crisis aid or disorder-specific coaching. Evaluating all apps using the same functional rubric while ensuring consistency may obscure nuances specific to certain therapeutic niches. For example, an app not including psychoeducation may not reflect poor quality if its sole purpose is

to offer peer support or emotional journaling. Conversely, apps that include a high number of features may score well despite poor implementation or compromised user experience.

External validity represents another concern. While the study included 44 mental health apps, selection bias may have occurred: only English-language apps accessible in certain regions and platforms (iOS and Android) were analyzed. The rapidly evolving app ecosystem means that updates, feature rollouts or app removals could quickly render some assessments obsolete. Furthermore, user needs vary considerably across demographics and clinical conditions, and the scoring rubric, while structured, may not reflect culturally specific needs or considerations relevant to minority mental health populations (e.g., LGBTQ+ affirming care or trauma-informed approaches).

During the elicitation phase of the assessment questions, some important characteristics may have been omitted, therefore threatening internal and conclusion validity. However, this bias was substantially reduced through performing a thorough review of scientific literature and app repositories to select the most significant characteristics in relation to mental health care. Moreover, both the development of the assessment questionnaire and the evaluation of the apps selected were performed independently by the first two authors and reviewed by the remaining authors.

Lastly, threats to statistical conclusion validity arise in the exploration of correlations particularly between app store ratings and functional scores. As demonstrated in the earlier analysis, no strong linear relationship was found between average user rating and assessment score, but this may be due to confounding variables such as brand recognition, visual design appeal, pricing

models or biased review patterns. Ratings also do not necessarily reflect clinical effectiveness or ethical soundness, which may lead to over- or underestimation of quality when relying solely on marketplace data. Another threat to conclusion validity concerned the accuracy of the results obtained. To mitigate this issue, the findings were presented both textually and graphically. The external validity may have been biased by the unavailability of users' ratings for certain apps. This was addressed by only considering apps that provided users' ratings in the correlation analysis.

In summary, while the present study applies a structured and evidence-informed framework, limitations related to subjective interpretation, heterogeneity of app purpose, selection scope and evolving market conditions introduce potential threats to internal, external and construct validity. These must be acknowledged when generalizing or applying the findings to broader clinical or technological conclusions.

5. Conclusion and Future Work

Mental health disorders such as anxiety, depression and ADHD represent growing public health concerns that increasingly require scalable and accessible interventions. In this context, mobile mental health applications have emerged as potentially valuable self-help and therapeutic tools. The present study aimed to evaluate the functional and technical quality of 44 mental health apps across Android and iOS platforms using a comprehensive assessment framework composed of 38 criteria covering evidence-based clinical content, self-monitoring features, therapeutic support, crisis management, educational content and technical foundations such as usability, accessibility and privacy.

The findings to each of the research questions of the present study are as follows:

RQ1. Which are the common types of mental health care apps analyzed?

The apps reviewed primarily fall into several categories: those offering self-help tools such as journaling and mood tracking (approximately 40%), those providing guided therapeutic exercises based on CBT or mindfulness approaches (approximately 35%), those facilitating connections with professionals or peer communities (approximately 20%), and those focused primarily on meditation and relaxation practices (approximately 5%). Apps targeting specific disorders such as ADHD or PTSD remain underrepresented compared to those addressing general anxiety and depression.

RQ2. Which are the predominant functional characteristics of mental health care apps?

The most common features include guided CBT exercises (87% of apps), mindfulness sessions (80%), and mood tracking (84%). However, more advanced functionalities such as cognitive restructuring tools, exposure tasks or structured therapy modules were present in fewer apps. Peer support mechanisms and direct access to professionals were relatively rare, appearing in fewer than 20% of apps.

RQ3. To what extent do the apps comply with the identified features?

App compliance varied widely. While a few apps such as AIME Mental Health and Wellbeing, Wysa and DBT Coach covered a broad range of evidence-based and technical criteria, many others despite achieving high app store ratings lacked clinical depth or failed to implement essential features such as crisis support or comprehensive educational content. Most apps performed best in engagement-related areas such as tracking and reminders, but scored lower on structured interventions, professional support access and emergency preparedness.

RQ4. Are user ratings proportionally linked to the apps' characteristics?

No significant correlation was found between user ratings and assessment scores ($r = -0.048$, $p = .753$). Several apps with high store ratings (e.g., Dare, Anxiety Test & Relief) received low functional scores due to limited evidence-based content. Conversely, apps with strong assessment scores were sometimes rated lower by users, possibly due to usability concerns or performance issues. This suggests that store ratings alone are insufficient to assess therapeutic quality.

RQ5. Which mental health disorders are most frequently targeted by these apps, and how are they addressed?

Anxiety and depression were the most frequently targeted disorders, addressed through CBT exercises, relaxation techniques and mindfulness content. ADHD, PTSD and OCD were addressed in only a few specialized apps. Moreover, the approaches used were often generalized rather than tailored to specific diagnostic needs, limiting their potential effectiveness for certain populations. Serious mental illnesses such as bipolar disorder, schizophrenia or eating disorders received minimal attention.

The findings of the present study denote the need to develop mobile solutions that cater to a broader range of mental health conditions beyond anxiety and depression, particularly addressing gaps in crisis management, professional support access and disorder-specific interventions. Moreover, developers are required to minimize usability flaws and strengthen security measures. In this regard, the present study can provide a baseline for developing qualified and effective apps that are suited to the diverse needs of individuals seeking mental health support.

As future work, it is intended to comprehensively measure the compliance of mental health care apps with usability guidelines and quality characteristics using established quality assessment tools (e.g., Mobile App Rating Scale MARS) and international standards (e.g., ISO/IEC 25010). Being particularly important in facilitating user experience and promoting efficiency, effectiveness and user satisfaction, usability is widely recognized as a key quality factor in the development of successful interactive software applications.

Another foreseen investigation consists of developing a reusable requirements catalog for designing well-suited and complete mental health care apps. Previous studies have established catalogs of reusable requirements for mobile health applications covering usability, sustainability

and internationalization. Such work could be extended to the mental health domain to establish comprehensive guidelines encompassing functional suitability, usability, and security requirements. Given the privacy and security concerns identified in the present study, particular attention should be devoted to data protection, encryption standards, and regulatory compliance within such catalogs.

Building upon requirements specification, future research should investigate the development and validation of evidence-based interventions targeting specific mental health conditions. By translating identified best practices into prototype applications designed according to systematic requirements engineering principles, researchers can establish the clinical effectiveness of theory-driven digital interventions through randomized controlled trials. Longitudinal studies examining user engagement patterns, adherence rates and clinical outcomes would provide valuable insights into the real-world impact of mental health apps. Furthermore, exploring the integration of mental health apps within traditional care pathways represents an important area for future investigation.

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APPENDIX A

Table A.1: Characteristics of the 44 apps evaluated for mental health care

App Name	OS	Category	Latest Update	Cost Model	URL	Country	Features	Google Rating	App Store Rating
7 Cups: Therapy & Support	Android & iOS	Health & Fitness	12-Jan-2026	Freemium	https://www.7cups.com	USA	Provides 24/7 emotional support through volunteer listeners and licensed therapists along with guided wellness paths.	3.5	4.4
AIME Mental Health & Wellbeing	Android & iOS	Health & Fitness	15-Nov-2025	Freemium	https://aime.health	UK	Offers personalized well-being strategies and daily check-ins to monitor mental health progress.	3.9	5
ACT Companion	Android & iOS	Health & Fitness	10-Jan-2026	Subscription	https://www.actcompanion.com	Australia	Provides Acceptance and Commitment Therapy (ACT) tools including	3.9	4.3

							mindfulness and values tracking.		
Anxiety Test & Relief	Android & iOS	Medical	20-Dec-2025	Freemium	https://mcreynolds.app	USA	Includes clinical anxiety assessments and stress management tools for progress tracking.	4.2	4.6
Beyond MST	Android & iOS	Medical	04-Jan-2026	Free	https://www.ptsd.va.gov	USA	Specifically designed for survivors of Military Sexual Trauma with education and coping resources.	4.6	4.8
CBT Companion: Therapy Journal	Android & iOS	Medical	12-Dec-2025	Freemium	https://cbtcompanion.com	USA	A digital CBT thought diary featuring video lessons and cognitive distortion identification tools.	3.9	4.6
CBT Therapy – Sintelly	Android & iOS	Health & Fitness	05-Jan-2026	Freemium	https://sintelly.com	Croatia	Features psychological tests and CBT exercises supported by a community forum.	4.4	4.5

Claria: Anxiety & Depression	Android & iOS	Health & Fitness	20-Nov-2025	Subscription	https://claria.care	USA	Structured CBT and ACT programs with guided meditations for managing anxiety and mood.	4.3	4.6
Clarity: CBT Self Help Journal	Android & iOS	Health & Fitness	05-Jan-2026	Freemium	https://clarityapp.io	USA	Focuses on CBT thought recording and identifying patterns in emotional health via journaling.	4.6	4.9
Dare: Anxiety & Panic Attacks	Android & iOS	Health & Fitness	09-Jan-2026	Freemium	https://daresponse.com	Ireland	Uses specialized audio guides and an SOS button to help users overcome panic attacks.	4.8	4.9
DBT Coach	Android & iOS	Health & Fitness	15-Dec-2025	Freemium	https://dbtcoach.com	USA	Comprehensive DBT tools including diary cards skill tracking and behavioral analysis.	4.4	4.7
Dr.Listen	Android	Health & Fitness	02-Jan-2026	Freemium	https://drlisten.ai	South Korea	AI-driven mental health analysis and	4.3	4.7

	& iOS						personalized emotional coaching through assessments.		
Flow – Depression treatment	Android & iOS	Medical	10-Dec-2025	Subscription	https://flowneuroscience.com	Sweden	CBT-based depression treatment program designed to be used alongside a brain stimulation headset.	4.5	4.8
gonna be ok	Android & iOS	Health & Fitness	12-Nov-2025	Free	https://gonnabeok.app	Italy	Provides daily mood tracking and coping strategies with a focus on simple self-care.	4.2	4.5
HappyMind: Mental Health	Android & iOS	Health & Fitness	05-Jan-2026	Freemium	https://happymind.app	USA	A cognitive-behavioral tool that focuses on positive psychology and habit building.	4.3	4.6
HarmLess: Self Harm Tracker	Android & iOS	Health & Fitness	01-Jan-2026	Free	https://harmless.org.uk	UK	Anonymized tracking tool for managing self-harm urges	4.4	4.6

							with distraction techniques.		
How We Feel	Android & iOS	Health & Fitness	18-Dec-2025	Free	https://howwefeel.org	USA	A science-based mood journal that helps users identify and regulate emotions with video lessons.	4.8	4.9
I'm Fine: Mental Health Guide	Android & iOS	Health & Fitness	15-Oct-2025	Freemium	https://im-fine.app	Romania	Personalized mental health assistant providing mood tracking and guided coping exercises.	4.2	4.5
Inflow – Manage your ADHD	Android & iOS	Health & Fitness	08-Jan-2026	Subscription	https://getinflow.io	UK	A CBT-based program for ADHD management featuring modules on focus and organization.	4.6	4.7
MendMe: CBT & Therapy	Android & iOS	Health & Fitness	20-Nov-2025	Freemium	https://mendme.app	Turkey	Therapeutic journal using CBT techniques to help manage stress and anxiety.	4.1	4.5

MindDoc: Mental Health Support	Android & iOS	Medical	11-Jan-2026	Freemium	https://minddoc.com	Germany	Monitoring tool for depression and anxiety with 14-day mental health assessments.	4.7	4.7
MindHealth: CBT Mental Health	Android & iOS	Health & Fitness	02-Dec-2025	Freemium	https://mindhealth.ai	USA	AI-powered mental health companion providing CBT-based interventions and mood tracking.	4.7	4.8
MindDay: mental health stress	Android & iOS	Health & Fitness	11-Jan-2026	Subscription	https://minddday.com	France	Self-therapy app offering video-based CBT sessions and stress reduction programs.	N/A	4.8
Moodfit: Mental Health Fitness	Android & iOS	Health & Fitness	15-Dec-2025	Freemium	https://getmoodfit.com	USA	Comprehensive fitness app for the mind featuring mood tracking and CBT thought records.	4.2	4.6
MyPossibleSelf:	Android	Health & Fitness	09-Jan-2026	Free	https://mypossibleself.com	UK	Offers various modules based on CBT to manage	4.3	4.7

Mental Health	& iOS						anxiety stress and improve wellness.		
Choiceful OCD Treatment App	Android & iOS	Health & Fitness	13-Jan-2026	Subscription	https://bechoiceful.com	USA	Specialized tool for OCD using exposure therapy and recovery planning.	4.5	4.7
Onsen – AI for Mental Health	Android & iOS	Health & Fitness	05-Jan-2026	Freemium	https://onsen.ai	Singapore	AI-driven journal and conversational assistant for emotional reflection and clarity.	4.65	N/A
Panic Attack Help – Mind Ease	Android & iOS	Health & Fitness	09-Jan-2026	Freemium	https://mindease.io	UK	Specialized in rapid relief for panic attacks with breathing tools and quick-calm exercises.	4.8	4.7
Psyche: Mental Health	Android & iOS	Health & Fitness	20-Dec-2025	Freemium	https://psyche.health	USA	CBT and mindfulness tools aimed at building mental resilience and self-awareness.	N/A	N/A

Psyhelp: Therapy & Psychology	Android & iOS	Health & Fitness	10-Nov-2025	Freemium	https://psyhelp.app	Turkey	Connects users with psychologists while providing CBT-based self-help tools.	N/A	N/A
PTSD Coach	Android & iOS	Medical	04-Jan-2026	Free	https://ptsd.va.gov	USA	Evidence-based tool for managing PTSD symptoms with tracking and crisis support.	4.6	4.8
Rootd – Anxiety & Panic Relief	Android & iOS	Health & Fitness	03-Jan-2026	Freemium	https://rootd.io	Canada	Focuses on immediate panic attack relief with the "Rootr" button and anxiety education.	4.5	4.8
Self Therapy journey: Relate	Android & iOS	Health & Fitness	12-Dec-2025	Subscription	https://relate.app	USA	Provides guided therapeutic journeys for relationships using psychological models.	4.6	4.7
Talkspace Go	Android & iOS	Health & Fitness	20-Nov-2025	Subscription	https://talkspace.com	USA	Self-guided therapy app offering workshops and assessments to	4	4.4

							complement therapy.		
therapini – affordable therapy	Android & iOS	Health & Fitness	05-Jan-2026	Subscription	https://therapini.com	USA	Platform for online therapy and CBT-based mental health tracking.	N/A	N/A
ThoughtFull: Mental Health App	Android & iOS	Health & Fitness	08-Jan-2026	Subscription	https://thoughtfull.world	Singapore	Provides 1-on-1 text coaching and daily wellness tools.	N/A	N/A
UpLife: Mental Health Therapy	Android & iOS	Health & Fitness	15-Dec-2025	Subscription	https://uplife.app	Cyprus	Psychology-based courses and CBT techniques focused on habit building.	4.4	4.6
UpLift – Depression & Anxiety	Android & iOS	Health & Fitness	10-Jan-2026	Subscription	https://uplift.app	USA	Science-backed treatment for depression with interactive CBT modules.	N/A	4.9
Voda: LGBTQIA+ Mental Wellness	Android & iOS	Health & Fitness	12-Jan-2026	Subscription	https://voda.co	UK	Queer-led mental health support focusing on intersectionality and CBT.	4.1	4.3

Worry Watch: Anxiety & Mood	Android & iOS	Health & Fitness	05-Jan-2026	Subscription	https://worrywatch.com	India	Advanced anxiety tracking and data visualization tool with secure passcode protection.	N/A	N/A
WorryTree: Anxiety CBT Therapy	Android & iOS	Health & Fitness	15-Nov-2025	Freemium	https://worry-tree.com	UK	A worry management tool using cognitive restructuring and action planning.	4	4.9
Wysa: Anxiety	Android & iOS	Health & Fitness	07-Jan-2026	Freemium	https://wysa.io	India	AI-powered chatbot for CBT and mindfulness with professional coaching options.	4.7	4.9
Thoughts (CBT Thought Diary)	Android & iOS	Health & Fitness	10-Jan-2026	Freemium	https://cbtthoughtdiary.com	Canada	Digital thought diary for identifying cognitive distortions and tracking mood trends.	N/A	4.7
SanaAI	Android	Health & Fitness	05-Jan-2026	Freemium	https://sanaai.com	Sweden	AI-driven mental health coaching providing 24/7	N/A	4.6

	& iOS						personalized support.		

APPENDIX B

Table B.1: Assessment Results (Part 1) Clinical Content & Self-Monitoring (CC1-CC10)

Assessment questions	CC1	CC2	CC3	CC4	CC5	CC6	CC7	CC8	CC9	CC10
7 Cups: Therapy & Support	Y	Y	N	Y	N	N	N	N	Y	Y
AIME Mental Health & Wellbeing	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y

ACT Companion	N	Y	Y	Y	N	Y	N	Y	N	Y
Anxiety Test & Relief	N	N	N	N	N	N	N	N	N	N
Beyond MST	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
CBT Companion: Therapy Journal	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
CBT Therapy - Sintelly	Y	Y	N	Y	N	Y	Y	Y	Y	Y
Claria: Anxiety & Depression	Y	Y	N	Y	N	N	Y	Y	Y	Y
Clarity: CBT Self Help Journal	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
Dare: Anxiety & Panic Attacks	Y	Y	Y	Y	Y	Y	Y	Y	Y	N
DBT Coach	Y	Y	Y	Y	Y	Y	Y	Y	N	Y
Dr.Listen	Y	N	N	N	Y	Y	N	N	Y	Y
Flow - Depression treatment	Y	Y	Y	Y	N	N	Y	Y	Y	Y
gonna be ok mental health care	Y	Y	N	Y	Y	Y	N	N	Y	Y
HappyMind: Mental Health	Y	N	N	N	Y	Y	Y	Y	Y	Y
HarmLess: Self Harm Tracker	Y	Y	N	Y	Y	Y	Y	Y	N	Y
How We Feel	N	Y	Y	Y	N	Y	Y	Y	N	Y
I'm Fine: Mental Health Guide	N	Y	N	Y	N	N	N	N	N	Y
Inflow - Manage your ADHD	Y	Y	N	Y	Y	Y	Y	Y	Y	Y

MendMe: CBT & Therapy	Y	Y	N	Y	N	Y	Y	Y	Y	Y
MindDoc: Mental Health Support	Y	Y	N	Y	Y	Y	Y	Y	Y	Y
MindHealth: CBT Mental Health	Y	N	N	N	Y	Y	Y	Y	Y	Y
MindDay: mental health stress	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
Moodfit: Mental Health Fitness	Y	Y	N	Y	Y	Y	Y	Y	Y	Y
MyPossibleSelf: Mental Health	Y	Y	N	Y	Y	Y	Y	Y	Y	Y
Choiceful OCD treatment App	Y	N	Y	N	Y	Y	Y	Y	Y	Y
Onsen – AI for Mental Health	Y	Y	N	Y	N	Y	Y	Y	Y	N
Panic Attack Help - Mind Ease	Y	Y	N	Y	Y	Y	N	Y	Y	Y
Psyche: Mental Health	Y	Y	N	Y	Y	Y	N	Y	Y	Y
Psyhelp: Therapy & Psychology	Y	Y	Y	Y	Y	Y	Y	Y	Y	N
PTSD Coach	N	Y	N	Y	N	Y	N	Y	Y	Y
Rootd - Anxiety & Panic Relief	Y	Y	N	Y	N	Y	Y	Y	Y	N
Self Therapy journey: Relate	Y	Y	N	Y	Y	Y	Y	Y	Y	Y
Talkspace Go	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
therapini - affordable therapy	Y	Y	Y	Y	Y	Y	N	N	N	N

ThoughtFull: Mental Health App	Y	N	N	N	N	Y	Y	N	N	Y
UpLife: Mental Health Therapy	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
UpLift - Depression & Anxiety	Y	N	N	N	N	N	Y	Y	Y	Y
Voda: LGBTQIA+ Mental Wellness	Y	Y	Y	Y	N	N	Y	Y	N	Y
Worry Watch: Anxiety & Mood	Y	Y	N	N	N	N	N	Y	N	N
WorryTree: Anxiety CBT Therapy	Y	Y	N	Y	N	Y	Y	Y	N	Y
Wysa: Anxiety	Y	Y	Y	Y	Y	Y	Y	Y	Y	Y
Thoughts	Y	N	N	N	N	Y	N	N	N	N
SanaAI	Y	Y	N	Y	Y	Y	Y	Y	Y	Y
Total (N)	39	36	17	35	26	36	32	36	32	36
Total (%)	86.67%	80.00%	37.78%	77.78%	57.78%	80.00%	71.11%	80.00%	71.11%	80.00%

Table B.2: Assessment Results (Part 2) Self-Monitoring & Therapy (SMR12-SMR16)

Assessment questions	SMR11	SMR12	SMR13	SMR14	SMR15	SMR16
7 Cups: Therapy & Support	Y	Y	N	N	N	N
AIME Mental Health & Wellbeing	Y	Y	Y	Y	Y	Y
ACT Companion	N	Y	Y	Y	N	Y
Anxiety Test & Relief	Y	N	N	N	N	Y
Beyond MST	Y	Y	Y	Y	Y	Y

CBT Companion: Therapy Journal	Y	Y	Y	Y	N	Y
CBT Therapy - Sintelly	Y	Y	N	N	Y	Y
Claria: Anxiety & Depression	Y	Y	N	N	N	Y
Clarity: CBT Self Help Journal	Y	Y	N	Y	Y	Y
Dare: Anxiety & Panic Attacks	N	N	N	N	N	N
DBT Coach	Y	Y	N	Y	Y	Y
Dr.Listen	Y	Y	Y	Y	Y	Y
Flow - Depression treatment	Y	Y	Y	N	N	Y
gonna be ok mental health care	Y	Y	Y	Y	Y	Y
HappyMind: Mental Health	Y	N	N	N	N	N
HarmLess: Self Harm Tracker	Y	Y	N	Y	N	Y
How We Feel	Y	Y	N	Y	Y	Y
I'm Fine: Mental Health Guide	Y	Y	N	N	Y	Y
Inflow - Manage your ADHD	N	N	Y	Y	Y	Y
MendMe: CBT & Therapy	Y	Y	Y	N	N	Y
MindDoc: Mental Health Support	Y	Y	N	N	Y	Y
MindHealth: CBT Mental Health	Y	Y	N	Y	N	Y
MindDay: mental health stress	Y	Y	N	Y	Y	Y
Moodfit: Mental Health Fitness	Y	Y	Y	Y	Y	Y
MyPossibleSelf: Mental Health	Y	Y	Y	Y	Y	Y
Choiceful OCD treatment App	N	Y	N	N	Y	N
Onsen – AI for Mental Health	Y	Y	N	N	Y	Y
Panic Attack Help - Mind Ease	Y	Y	N	N	Y	Y

Psyche: Mental Health	N	N	N	N	N	N
Psyhelp: Therapy & Psychology	Y	Y	N	Y	Y	Y
PTSD Coach	Y	N	Y	Y	N	Y
Rootd - Anxiety & Panic Relief	Y	Y	Y	Y	N	Y
Self Therapy journey: Relate	Y	Y	N	N	Y	Y
Talkspace Go	N	Y	Y	Y	N	N
therapini - affordable therapy	Y	Y	N	Y	N	N
ThoughtFull: Mental Health App	Y	Y	N	N	Y	Y
UpLife: Mental Health Therapy	Y	Y	N	N	Y	Y
UpLift - Depression & Anxiety	Y	Y	N	N	Y	Y
Voda: LGBTQIA+ Mental Wellness	Y	Y	N	N	Y	Y
Worry Watch: Anxiety & Mood	Y	Y	N	Y	Y	Y
WorryTree: Anxiety CBT Therapy	Y	Y	Y	Y	Y	Y
Wysa: Anxiety	Y	Y	Y	Y	Y	Y
Thoughts	Y	Y	N	N	N	N
SanaAI	Y	Y	Y	N	Y	Y
Total (N)	38	38	17	23	27	36
Total (%)	84.44%	84.44%	37.78%	51.11%	60.00%	80.00%

Table B.3: Assessment Results (Part 3) Therapy, Counseling, and Peer Support (TCS17 - TCS23)

Assessment questions	TCS17	TCS18	TCS19	TCS20	TCS21	TCS22	TCS23

7 Cups: Therapy & Support	Y	Y	Y	Y	Y	Y	Y
AIME Mental Health & Wellbeing	Y	Y	Y	Y	N	Y	Y
ACT Companion	N	Y	N	N	N	N	N
Anxiety Test & Relief	N	N	N	N	N	N	N
Beyond MST	Y	Y	Y	N	N	N	Y
CBT Companion: Therapy Journal	N	Y	N	N	N	N	N
CBT Therapy - Sintelly	N	Y	N	N	N	N	N
Claria: Anxiety & Depression	N	Y	N	N	N	N	N
Clarity: CBT Self Help Journal	N	Y	N	N	N	N	Y
Dare: Anxiety & Panic Attacks	N	Y	Y	Y	N	N	N
DBT Coach	N	Y	Y	Y	N	Y	N
Dr.Listen	N	N	N	N	N	N	N
Flow - Depression treatment	N	Y	N	N	N	N	N
gonna be ok mental health care	N	Y	Y	Y	N	Y	Y
HappyMind: Mental Health	N	Y	N	N	N	N	Y
HarmLess: Self Harm Tracker	N	Y	N	N	Y	N	N
How We Feel	N	N	N	N	N	N	N
I'm Fine: Mental Health Guide	N	N	N	N	Y	N	Y
Inflow - Manage your ADHD	N	Y	Y	Y	N	Y	Y
MendMe: CBT & Therapy	N	Y	N	N	N	N	N
MindDoc: Mental Health	N	Y	N	N	Y	N	N

Support							
MindHealth: CBT Mental Health	N	Y	N	N	N	N	N
MindDay: mental health stress	N	Y	N	N	N	N	N
Moodfit: Mental Health Fitness	N	N	N	N	N	N	N
MyPossibleSelf: Mental Health	N	Y	N	N	N	N	N
Choiceful OCD treatment App	N	Y	N	N	N	Y	Y
Onsen – AI for Mental Health	N	Y	N	N	N	N	Y
Panic Attack Help - Mind Ease	N	Y	N	N	N	N	N
Psyche: Mental Health	N	Y	N	N	N	N	N
Psyhelp: Therapy & Psychology	N	Y	Y	Y	N	N	N
PTSD Coach	N	N	N	N	Y	N	N
Rootd - Anxiety & Panic Relief	N	Y	N	N	N	N	N
Self Therapy journey: Relate	N	Y	N	N	N	N	N
Talkspace Go	Y	Y	N	Y	Y	N	Y
therapini - affordable therapy	N	N	N	N	N	N	Y
ThoughtFull: Mental Health App	Y	N	N	N	N	Y	N
UpLife: Mental Health Therapy	N	Y	N	N	N	N	Y
UpLift - Depression & Anxiety	N	Y	N	N	N	N	N
Voda: LGBTQIA+ Mental Wellness	N	Y	N	N	Y	N	N
Worry Watch: Anxiety & Mood	N	N	N	N	N	N	N

WorryTree: Anxiety CBT Therapy	N	N	N	N	N	N	N
Wysa: Anxiety	Y	Y	N	N	Y	N	Y
Thoughts	N	N	N	N	N	N	Y
SanaAI	N	Y	Y	Y	N	Y	Y
Total (N)	6	33	9	9	8	8	16
Total (%)	13.33%	73.33%	20.00%	20.00%	17.78%	17.78%	35.56%

Table B.4: Assessment Results (Part 4) Crisis Management and Emergency Aid (CMEA24 - CMEA26)

Assessment questions	CMEA24	CMEA25	CMEA26
7 Cups: Therapy & Support	N	N	N
AIME Mental Health & Wellbeing	N	N	N
ACT Companion	N	N	Y
Anxiety Test & Relief	N	N	N
Beyond MST	Y	Y	N
CBT Companion: Therapy Journal	N	N	N
CBT Therapy - Sintelly	N	N	N
Claria: Anxiety & Depression	N	N	N
Clarity: CBT Self Help Journal	N	N	N
Dare: Anxiety & Panic Attacks	N	N	Y
DBT Coach	Y	Y	Y

Dr.Listen	N	N	N
Flow - Depression treatment	Y	N	N
gonna be ok mental health care	N	N	N
HappyMind: Mental Health	N	N	N
HarmLess: Self Harm Tracker	Y	Y	N
How We Feel	Y	Y	N
I'm Fine: Mental Health Guide	N	N	N
Inflow - Manage your ADHD	N	N	N
MendMe: CBT & Therapy	N	N	N
MindDoc: Mental Health Support	Y	Y	N
MindHealth: CBT Mental Health	N	N	N
MindDay: mental health stress	N	N	N
Moodfit: Mental Health Fitness	N	N	N
MyPossibleSelf: Mental Health	Y	Y	N
Choiceful OCD treatment App	N	N	N
Onsen – AI for Mental Health	N	N	N
Panic Attack Help - Mind Ease	N	N	Y
Psyche: Mental Health	Y	N	N
Psyhelp: Therapy & Psychology	N	N	N
PTSD Coach	Y	Y	Y
Rootd - Anxiety & Panic Relief	N	Y	Y
Self Therapy journey: Relate	N	N	N

Talkspace Go	Y	Y	N
therapini - affordable therapy	Y	N	N
ThoughtFull: Mental Health App	Y	Y	Y
UpLife: Mental Health Therapy	N	N	N
UpLift - Depression & Anxiety	N	N	N
Voda: LGBTQIA+ Mental Wellness	Y	Y	N
Worry Watch: Anxiety & Mood	N	N	Y
WorryTree: Anxiety CBT Therapy	N	N	N
Wysa: Anxiety	Y	Y	Y
Thoughts	N	N	N
SanaAI	N	N	N
Total (N)	14	12	9
Total (%)	31.11%	26.67%	20.00%

Table B.5: Assessment Results (Part 5) Education and Personalized Content (EPC27 - EPC31)

Assessment questions	EPC27	EPC29	EPC30	EPC31
7 Cups: Therapy & Support	Y	N	Y	Y
AIME Mental Health & Wellbeing	Y	Y	Y	Y

ACT Companion	N	Y	Y	Y
Anxiety Test & Relief	N	N	N	N
Beyond MST	Y	Y	N	Y
CBT Companion: Therapy Journal	Y	N	Y	Y
CBT Therapy - Sintelly	Y	Y	N	Y
Claria: Anxiety & Depression	Y	N	N	N
Clarity: CBT Self Help Journal	Y	Y	Y	Y
Dare: Anxiety & Panic Attacks	Y	Y	Y	N
DBT Coach	Y	N	Y	Y
Dr.Listen	N	N	N	Y
Flow - Depression treatment	Y	N	Y	Y
gonna be ok mental health care	Y	N	Y	Y
HappyMind: Mental Health	Y	Y	Y	N
HarmLess: Self Harm Tracker	Y	Y	Y	N
How We Feel	Y	Y	N	N
I'm Fine: Mental Health Guide	Y	N	N	Y

Inflow - Manage your ADHD	Y	Y	Y	Y
MendMe: CBT & Therapy	Y	Y	Y	Y
MindDoc: Mental Health Support	Y	Y	Y	Y
MindHealth: CBT Mental Health	Y	Y	Y	Y
MindDay: mental health stress	Y	Y	Y	Y
Moodfit: Mental Health Fitness	Y	Y	Y	Y
MyPossibleSelf: Mental Health	Y	Y	Y	Y
Choiceful OCD treatment App	Y	Y	Y	N
Onsen – AI for Mental Health	Y	Y	Y	Y
Panic Attack Help - Mind Ease	Y	Y	N	Y
Psyche: Mental Health	N	N	Y	Y
Psyhelp: Therapy & Psychology	Y	Y	Y	Y
PTSD Coach	Y	N	N	Y
Rootd - Anxiety & Panic Relief	Y	Y	N	N
Self Therapy journey: Relate	Y	Y	Y	Y

Talkspace Go	Y	Y	Y	N
therapini - affordable therapy	N	N	N	Y
ThoughtFull: Mental Health App	Y	N	N	Y
UpLife: Mental Health Therapy	Y	Y	Y	Y
UpLift - Depression & Anxiety	Y	Y	N	Y
Voda: LGBTQIA+ Mental Wellness	Y	Y	Y	Y
Worry Watch: Anxiety & Mood	N	N	Y	Y
WorryTree: Anxiety CBT Therapy	N	N	N	Y
Wysa: Anxiety	Y	Y	Y	Y
Thoughts	Y	N	Y	N
SanaAI	N	Y	Y	Y
Total (N)	36	28	30	34
Total (%)	80.00%	62.22%	66.67%	75.56%

Table B.6: Assessment Results (Part 6) Technical Foundations & Total Scores (TF29-TF38 + Totals)

Assessment questions	TF31	TF32	TF33	TF314	TF35	TF36	TF37	TF38	**Total (N)**	**Total (%)**

7 Cups: Therapy & Support	N	Y	N	Y	Y	N	Y	Y	22	57.89%
AIME Mental Health & Wellbeing	N	Y	Y	Y	Y	Y	Y	Y	33	86.84%
ACT Companion	N	N	N	N	N	N	N	Y	16	42.11%
Anxiety Test & Relief	N	Y	Y	N	N	N	Y	Y	6	15.79%
Beyond MST	Y	Y	Y	Y	Y	Y	Y	Y	33	86.84%
CBT Companion: Therapy Journal	Y	Y	N	Y	N	Y	Y	Y	25	65.79%
CBT Therapy - Sintelly	N	Y	N	Y	N	N	Y	Y	20	52.63%
Claria: Anxiety & Depression	N	Y	N	Y	N	N	N	Y	15	39.47%
Clarity: CBT Self Help Journal	Y	Y	Y	N	N	N	N	Y	25	65.79%
Dare: Anxiety & Panic Attacks	N	Y	Y	N	N	N	Y	Y	20	52.63%
DBT Coach	Y	Y	N	Y	N	Y	Y	Y	30	78.95%
Dr.Listen	Y	Y	N	Y	N	N	Y	Y	17	44.74%
Flow - Depression treatment	N	Y	N	N	Y	Y	Y	Y	22	57.89%
gonna be ok mental health care	Y	Y	N	N	Y	N	N	Y	25	65.79%
HappyMind: Mental Health	N	N	N	N	N	N	Y	Y	15	39.47%
HarmLess: Self Harm Tracker	Y	Y	Y	Y	N	Y	Y	Y	26	68.42%

How We Feel	Y	Y	Y	Y	Y	Y	Y	Y	24	63.16%
I'm Fine: Mental Health Guide	N	N	N	Y	N	N	Y	Y	14	36.84%
Inflow - Manage your ADHD	N	Y	N	N	N	N	Y	Y	25	65.79%
MendMe: CBT & Therapy	N	Y	N	N	N	N	Y	Y	20	52.63%
MindDoc: Mental Health Support	N	Y	N	Y	Y	Y	Y	Y	27	71.05%
MindHealth: CBT Mental Health	N	Y	N	Y	Y	N	Y	Y	21	55.26%
MindDay: mental health stress	N	N	N	Y	N	N	Y	Y	23	60.53%
Moodfit: Mental Health Fitness	N	Y	N	N	N	Y	N	Y	22	57.89%
MyPossibleSelf: Mental Health	Y	Y	N	Y	Y	N	Y	Y	28	73.68%
Choiceful OCD treatment App	N	N	N	Y	N	N	N	Y	18	47.37%
Onsen – AI for Mental Health	Y	N	N	Y	Y	Y	Y	Y	23	60.53%
Panic Attack Help - Mind Ease	N	N	N	N	N	N	Y	Y	19	50.00%
Psyche: Mental Health	N	N	N	N	N	N	N	Y	13	34.21%
Psyhelp: Therapy & Psychology	N	Y	N	Y	N	N	Y	Y	25	65.79%
PTSD Coach	N	Y	Y	N	N	N	Y	Y	20	52.63%

Rootd - Anxiety & Panic Relief	N	N	Y	Y	Y	N	N	Y	21	55.26%
Self Therapy journey: Relate	N	Y	N	Y	N	Y	N	Y	22	57.89%
Talkspace Go	N	N	N	N	N	N	Y	Y	25	65.79%
therapini - affordable therapy	N	Y	N	Y	Y	N	N	Y	16	42.11%
ThoughtFull: Mental Health App	N	N	N	Y	N	N	Y	Y	18	47.37%
UpLife: Mental Health Therapy	N	Y	N	N	N	N	Y	Y	23	60.53%
UpLift - Depression & Anxiety	N	Y	N	N	N	N	N	Y	15	39.47%
Voda: LGBTQIA+ Mental Wellness	N	Y	N	N	N	N	Y	Y	22	57.89%
Worry Watch: Anxiety & Mood	Y	Y	Y	N	N	Y	Y	Y	17	44.74%
WorryTree: Anxiety CBT Therapy	N	Y	N	Y	Y	N	Y	Y	19	50.00%
Wysa: Anxiety	N	Y	N	N	Y	N	Y	Y	31	81.58%
Thoughts	N	N	N	Y	N	N	Y	Y	10	26.32%
SanaAI	N	Y	N	Y	Y	N	Y	Y	27	71.05%
Total (N)	11	32	10	25	15	12	33	44		
Total (%)	24.44%	71.11%	22.22%	55.56%	33.33%	26.67%	73.33%	97.78%		

APPENDIX C

Table C.1: User Ratings of Apps Selected

ID	App Name	OS	Ratings (Stars)	Total Raters
A1	7 Cups: Therapy & Support	Both	3.95	26,500
A2	AIME Mental Health & Wellbeing	Both	4.45	55
A3	ACT Companion	Both	4.1	-
A4	Anxiety Test & Relief	Both	4.4	-
A5	Beyond MST	Both	4.7	-
A6	CBT Companion: Therapy Journal	Both	4.25	-
A7	CBT Therapy – Sintelly	Both	4.45	-
A8	Claria: Anxiety & Depression	Both	4.45	-
A9	Clarity: CBT Self Help Journal	Both	4.75	-
A10	Dare: Anxiety & Panic Attacks	Both	4.85	-
A11	DBT Coach	Both	4.55	-
A12	Dr.Listen	Both	4.5	-
A13	Flow – Depression treatment	Both	4.65	-
A14	gonna be ok mental health care	Both	4.35	-
A15	HappyMind: Mental Health	Both	4.45	-
A16	HarmLess: Self Harm Tracker	Both	4.5	-
A17	How We Feel	Both	4.85	-
A18	I’m Fine: Mental Health Guide	Both	4.35	-
A19	Inflow – Manage your ADHD	Both	4.65	-

A20	MendMe: CBT & Therapy	Both	4.3	-
A21	MindDoc: Mental Health Support	Both	4.7	52,000
A22	MindHealth: CBT Mental Health	Both	4.75	5,370
A23	MindDay: mental health stress	iOS	4.8	175
A24	Moodfit: Mental Health Fitness	Both	4.4	11,000
A25	MyPossibleSelf: Mental Health	Both	4.5	1,200
A26	Choiceful OCD Treatment App	Both	4.6	75
A27	Onsen – AI for Mental Health	Android	4.65	195
A28	Panic Attack Help – Mind Ease	Both	4.75	21
A29	Psyche: Mental Health	Both	-	-
A30	Psyhelp: Therapy & Psychology	iOS	4.5	2
A31	PTSD Coach	Both	4.15	4,001
A32	Rootd – Anxiety & Panic Relief	Both	4.65	45,300
A33	Self Therapy journey: Relate	Both	4.65	1,500
A34	Talkspace Go	Both	4.7	8,950
A35	therapini – affordable therapy	Both	4.45	300
A36	ThoughtFull: Mental Health App	Both	4.35	350
A37	UpLife: Mental Health Therapy	Both	4.5	800
A38	UpLift – Depression & Anxiety	iOS	4.9	-
A39	Voda: LGBTQIA+ Mental Wellness	Both	4.2	369
A40	Worry Watch: Anxiety & Mood	Both	-	-
A41	WorryTree: Anxiety CBT Therapy	Both	4.45	8

A42	Wysa: Anxiety	Both	4.8	168,700
A43	Thoughts (CBT Thought Diary)	iOS	4.7	3
A44	SanaAI	iOS	4.6	84

Table C.2: Pearson Correlation Results

Variable	User's Rating	Assessment Score (%)
User's rating		
Pearson Corr.	1	-0.048
Sig. (2-tailed)	-	0.753
N	44	44
Assessment score		
Pearson Corr.	-0.048	1
Sig. (2-tailed)	0.753	-
N	44	44